

CENTRAL CALIFORNIA
EMERGENCY MEDICAL SERVICES
A Division of the Fresno County Department of Public Health

Manual	Emergency Medical Services Administrative Policies and Procedures	Policy Number 530.06
Subject	Paramedic Treatment Protocols DYSRHYTHMIAS	Page 1 of 2
References	Title 22, Division 9, Chapter 3.3 of the California Code of Regulations	Effective Fresno County: 01/15/82 Kings County: 04/10/89 Madera County: 06/15/85 Tulare County: 04/19/05

SPECIAL CONSIDERATIONS AND PRIORITIES

1. Assessment – Mental status, vital signs, lung sounds, neck vein distension, peripheral edema, pupils.
2. If unconscious or pulseless, proceed immediately to appropriate algorithms and stabilize before transport. In the setting of a life-threatening dysrhythmia with pulse, treat immediately and contact Base Hospital when indicated. Transport when indicated in algorithm. Consider early transport with treatment enroute.
3. Lights/siren transport is indicated for arrested or resuscitated patients. Non-lights/siren transport may be ordered when the risks of lights/siren outweigh the benefit to the patient.
4. Treat the patient and their signs/symptoms (e.g., level of consciousness, pulse, blood pressure), not the rhythm. The rhythm on the monitor must correlate with the presence or absence of a pulse.
5. Record a rhythm strip of patient's cardiac activity and submit it with the Prehospital Care Report (attach to back of top copy using scotch tape):
 - a. One strip of baseline rhythm.
 - b. Additional strips if rhythm interpretation is difficult, the rhythm changes, and before and after interventions, such as valsalva maneuver or electric shock.
6. Further evaluation:
 - a. Expand on the history of the chief complaint – O, P, Q, R, S, T

O: Onset
P: Provoked
Q: Quality
R: Radiation
S: Severity
T: Time

Approved By	Daniel J. Lynch	Revision
EMS Division Manager	(Signature on File at EMS Agency)	04/19/2005
EMS Medical Director	Jim Andrews, M.D. (Signature on File at EMS Agency)	

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- b. Medical history
- c. Medications
- d. Allergies
- e. Physical Exam

Emphasize: Symptomatic
Breath sounds
Skin color – temperature and moisture
Peripheral edema

- 7. CPR is indicated only if the patient is unconscious and pulseless, no matter what rhythm. A conscious adult patient has a better cardiac output than what could be achieved with CPR, even if no pulse is felt.
- 8. In the presence of isolated hypothermia, CPR is indicated for situations without an organized EKG rhythm (e.g., V-fib and asystole). Contact the Base Hospital regarding CPR on isolated hypothermia patients in PEA. Resuscitate until rewarming has been implemented in the emergency department.
- 9. Drug use (e.g., for behavioral emergencies) and medical conditions (e.g., diabetes) which effect metabolism may cause hypothermia even when outside temperatures may be temperate.